TEAM MEDVERSE | GATE 32

Bounded commitment under partial information

Decision

Timing is evaluated separately from authorization. COMMIT, bounded WAIT and authorized PREPARE use only released information. The unchanged Gate 29 selector and validator are shared by all methods; preparations cannot create care, disclosure or approval.

Design

4 development prefix families / 8 branches; 8 internal confirmation prefix families / 32 branches. 120 method runs. One common event-clock model; all branch variants remain grouped. No model training, new legal interpretation, or patient data.

Results

Eager commit: 14/32 complete, 0.00 mean proposal-wait steps; Fair bounded wait: 15/32 complete, 1.19 mean proposal-wait steps; MedVerse commitment: 15/32 complete, 1.19 mean proposal-wait steps.

Integrity

52/52 development checks; 177/177 identical-history checks; 72/72 change-before-commit probes without effects. 432/432 historical event outcomes preserved.

Interpretation

Against eager commitment, MedVerse improves 4 branches and loses 3. It ties the capable bounded-wait control on 32/32 branches. These are engineering results, not proof of unique superiority or optimal behavior under unknown futures.

Critical boundary

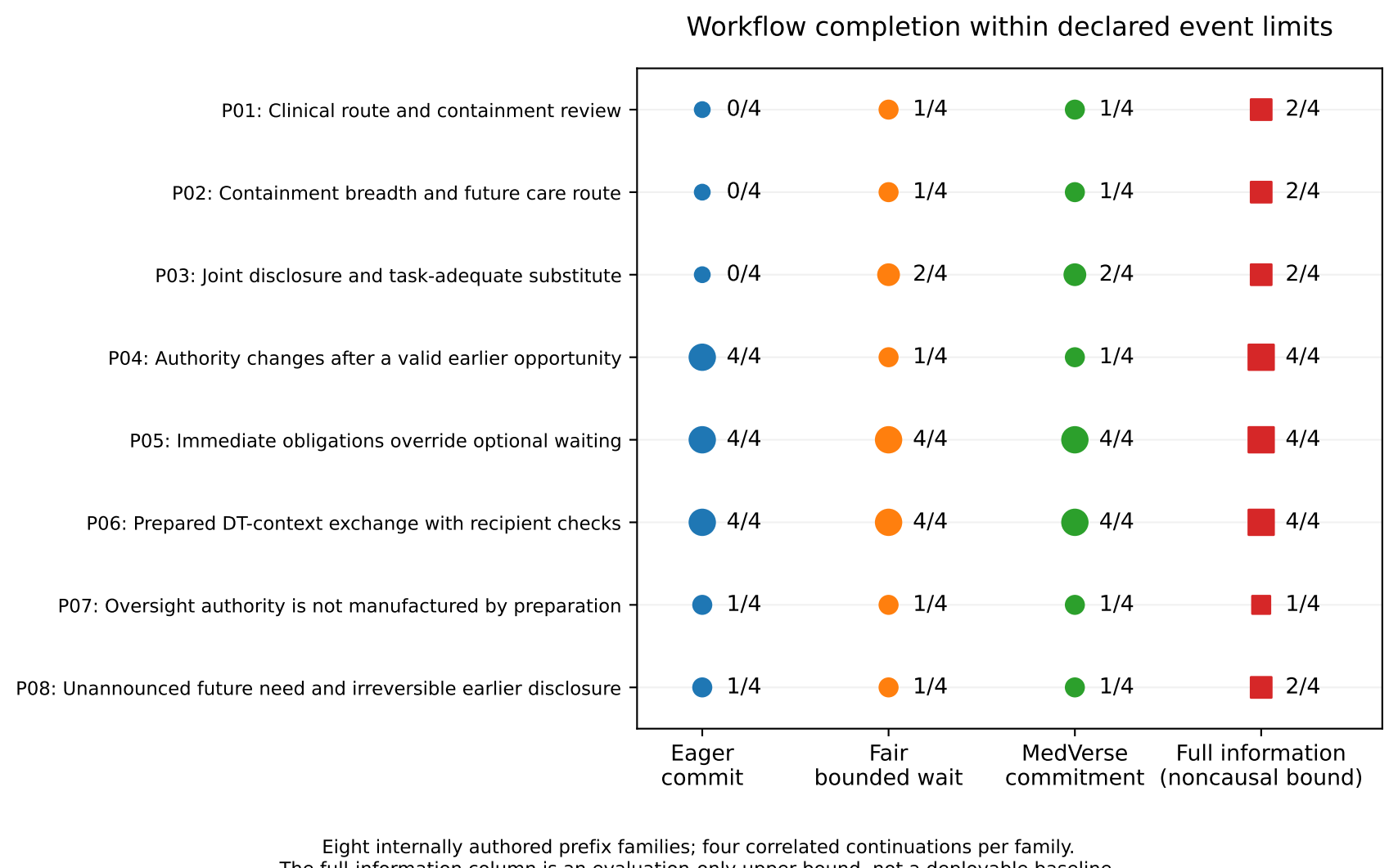
The old C01/C02 cases contain no declared review conditions justifying delay. They remain unchanged. New timing-contract evidence must not be presented as repairing those old cases. Earlier disclosures and commitments are never erased after later revocation.

Outstanding work

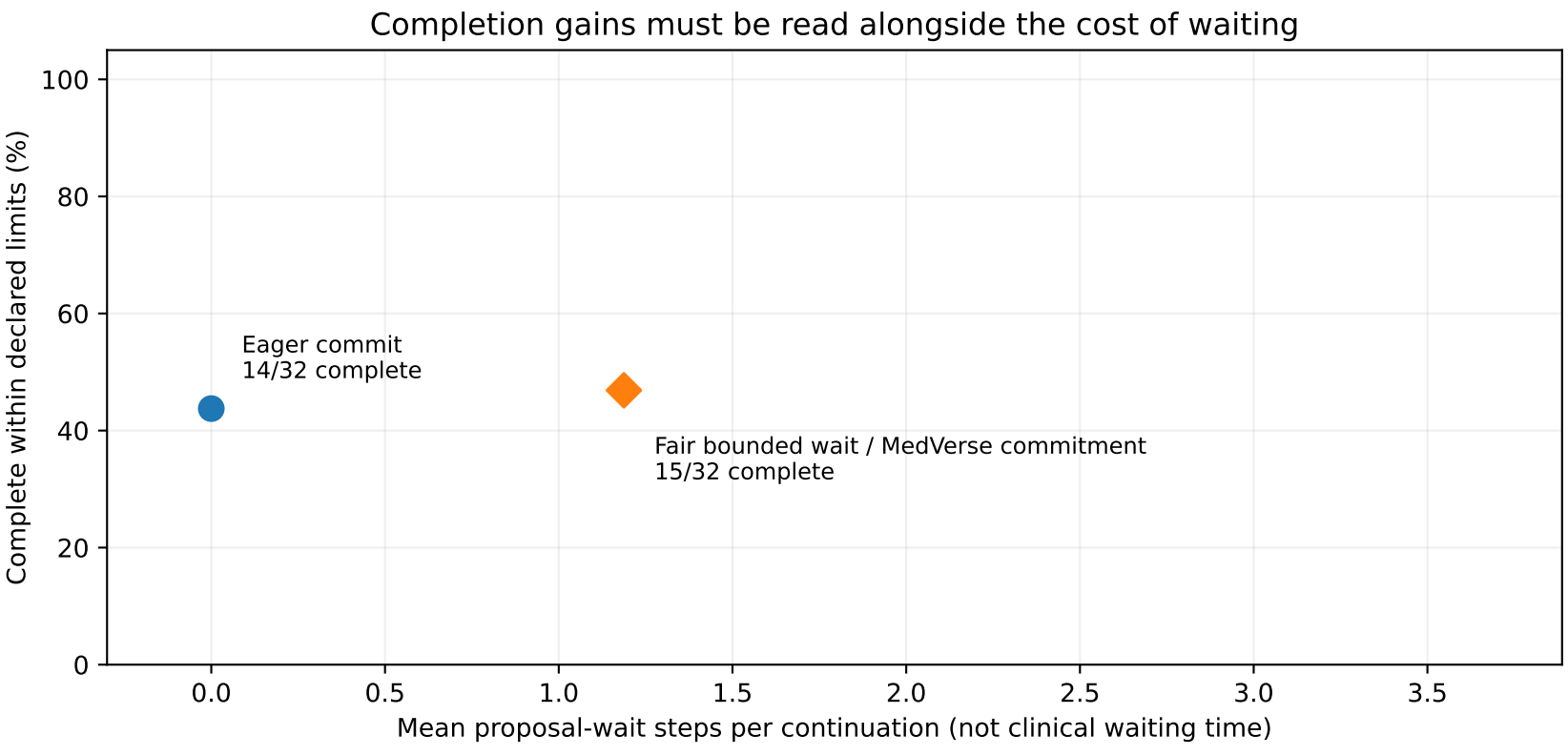
Qualified clinical/Korean-policy review, fresh independent scenario authorship, and integration of the tested commitment module into the final clinical workspace and video. The original DT/retrieval implementation is preserved; this experiment does not reload or reevaluate dense embeddings.

Reproduce

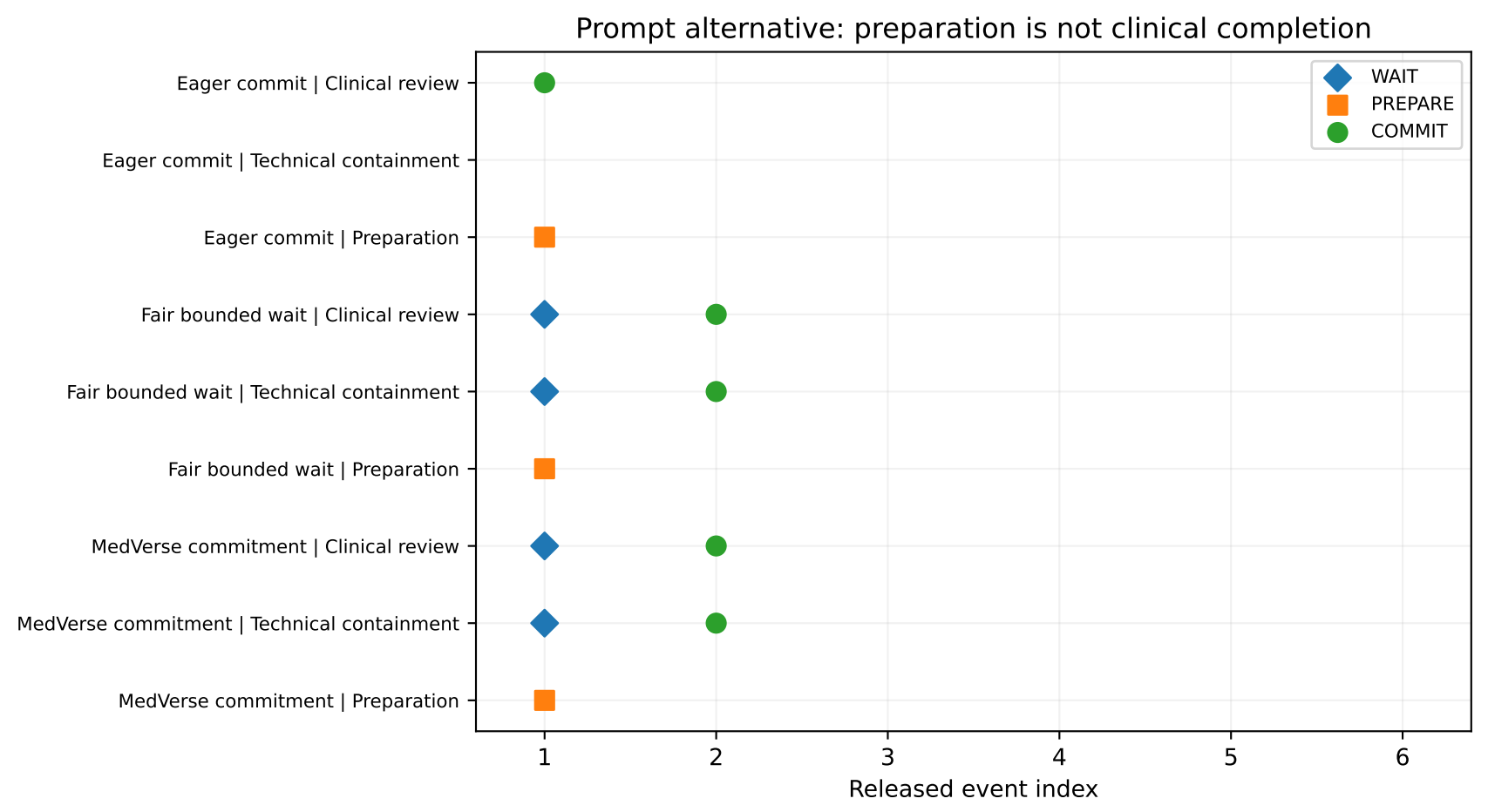
Run medverse/gates/gate32_commitment_control.py. It verifies frozen files, evaluates, saves evidence, exports PNG/PDF figures and this report, and displays the figures. Native Windows/Spyder execution is not independently tested here.



Eight internally authored prefix families; four correlated continuations per family.
The full-information column is an evaluation-only upper bound, not a deployable baseline.



Paired gains over eager: 4; losses: 3. Fair/MedVerse ties: 32/32.
Model event steps are declared engineering assumptions, not clinical timing recommendations.



Measured P01/PROMPT trace. Markers show actual permitted effects or recorded waits.
Other branches include late, absent and denied alternatives; the example is not the aggregate result.